Keep the Follow-Up Thread

A clinic buyer-hypothesis exercise

FICTIONAL / UNVERIFIED - SYNTHETIC EXERCISE ONLY

No real clinic, staff, or patient data. No buyer validation or outcome evidence.

The hypothesis-not a finding

Clinic staff **may** lose follow-up continuity between inquiry, booking, and visit.

The question is whether clearer ownership and a staff-reviewable workflow would fit existing work-not whether automation is automatically desirable.

Test ownership, workflow fit, and escalation needs before proposing a pilot.

A synthetic illustration only

Synthetic only - same synthetic reporting period

Inquiries	Booked	Visited	Associated revenue
86	62	41	USD 16400

These are supplied exercise figures, not clinic observations. No definitions, source records, or revenue attribution are available. They do not establish leakage, causation, or outcomes.

First understand today's workflow

1. What happens from first inquiry to booking?
2. Who owns each next step-and where does ownership change?
3. What happens after cancellation, no-show, or a visit?
4. Which tools and handoffs are already in use?
5. What makes follow-up hard, or already work well?

Use a recent example without patient-identifying details. Treat explanations as participant-reported until corroborated.

Staff review is a design requirement

- Staff must review and approve proposed outbound communications and workflow actions.
- Show the responsible role, context, and next step before approval.
- Keep an auditable human handoff; never imply that a draft was sent or an action completed without confirmation.
- Ask staff what must remain manual and what would make the workflow unacceptable.

No autonomous patient-facing or clinical decisions

Clinical escalation is explicit

- Ask the clinic to identify its existing escalation policy, responsible role, and safe route.
- Route clinical questions or concerning situations to qualified clinic staff under that policy.
- Do not generate clinical advice, triage, diagnosis, or urgency rules in this exercise.
- If the safe route is unclear, pause the workflow and resolve that before any pilot.

Unaided pricing questions

Ask before showing a price or suggesting a budget.

> "What would you pay for this, if anything? Please give a currency and amount. Zero is a valid answer, including USD 0."

Then ask what the amount covers, who would approve it, and what would need to be true. Record answers as unvalidated participant statements-not willingness-to-pay evidence.

Evidence boundary and next step

Available: only the approved synthetic brief. **Unavailable:** real workflow records, interviews, source definitions, attribution, market evidence, buyer validation, and outcomes.

Next: with authorization, interview staff and decision-makers; map roles and handoffs using fictional examples; verify review and escalation controls; seek disconfirming evidence.

Decision: consider a staff-supervised, narrowly scoped operational pilot only after workflow fit, ownership, privacy, and escalation are agreed. Otherwise revise or stop.